

NephroAssist — Pitch Deck

Transplant-Specific Patient Readiness & Care-Coordination Platform

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Slide 1: Problem — The Transplant Coordination Crisis

Transplant coordinators manage the most complex, high-stakes patient journey in medicine — with spreadsheets, phone trees, and sticky notes.

- **90,000+** kidney patients on the U.S. waiting list (SRTR/OPTN)
- **250+** transplant programs in the U.S. alone
- Coordinators juggle **50+ patients** across 5 distinct evaluation stages
- **10+ hours/week** lost chasing documents and reminding patients
- Missed appointments and incomplete evaluations **delay listing by weeks or months**
- **No dedicated tool exists** for pre-transplant patient readiness coordination

"We track everything in Excel. When a coordinator is out sick, nobody knows where anything stands." — Transplant Coordinator, Midwest Academic Medical Center

Slide 2: Solution — NephroAssist

The first transplant-specific readiness platform that sits between the EHR and the patient.

What it does: - Structured, organ-specific readiness checklists (kidney → heart, liver, lung) - Real-time patient-coordinator chat (not phone-only like CareDx) - Automated document collection and review workflows - Medication adherence reminders with care-team visibility - Patient education mapped to the 5-stage transplant journey

How it integrates: - SMART on FHIR launch from Epic/Cerner — read labs, appointments, demographics - Positioned as a **coordination layer**, not an EHR replacement - HIPAA-ready architecture; GDPR localization path for EU

Slide 3: Market — Niche, Sticky, Underserved

Metric	Value	Source
U.S. transplant programs	~250+	SRTR/OPTN
U.S. kidney waiting-list patients	~90,000	SRTR
Annual U.S. transplants	~40,000+	SRTR
EU+UK programs	~250–300	Eurotransplant + NHSBT
Annual EU+UK transplants	~17,000+	Eurotransplant + NHSBT

Addressable Market (U.S. only): - TAM at \$5K–\$50K/program/year: **\$1.25M–\$12.5M annually** - Expansion to EU+UK roughly doubles the program count - Post-transplant adherence, living-donor coordination, and pediatric modules extend TAM further

Why this market wins: - Sticky: transplant programs rarely switch tools mid-cycle - Mission-critical: listing delays directly affect patient survival - Currently served by EHR modules with known usability gaps

Slide 4: Product — Four Layers of Value

Layer 4: Intelligence & Analytics
– Coordinator dashboards, completion rates, time-to-listing metrics, SRTR reporting
Layer 3: Transplant Workflow Engine
– Organ-specific checklists, document review, approval chains, audit trails
Layer 2: Dialysis & Pre-Transplant Coordination
– Appointment scheduling, lab tracking, referral management, care-team chat
Layer 1: Patient Execution Layer
– Mobile PWA, task checklists, medication reminders, educational content, progress tracking

MVP scope (Months 1–6): Layer 1 + Layer 2 for kidney only **Phase 2 (Months 6–18):** Add heart, liver, lung modules; living-donor workflows **Phase 3 (Months 18–36):** AI-powered personalization, EU expansion, SRTTR automation

Slide 5: Competitive Landscape — The Gap Is Real

Competitor	Transplant-Specific?	Patient Checklist?	Real-Time Chat?	Weakness
CareDx	High (diagnostics)	No	Phone-only	No patient-facing readiness app
Transplant Hero	High (alarms only)	No	No	Alarm-only; no care team
iTransplant / InVita	High (logistics)	No	Yes (OPO teams)	Not patient-facing
SeamlessMD	Low	Yes	Partial	Not transplant-specific
Get Well Network	Low	Partial	Partial	Very broad; not transplant-specific
Mytonomy	Low	Partial	Partial	Education-heavy, light task mgmt
ThoroughCare	Low	Yes	No	Care-manager-centric, not patient-centric
NephroAssist	High	Yes	Yes	Early stage — first-mover in gap

Strategic positioning: - Against transplant incumbents: "We own pre-transplant coordination, not post-transplant diagnostics." - Against generic players: "They offer 50 care journeys. We offer one — and we know every step."

Slide 6: Business Model — Land-and-Expand B2B SaaS

Tier	Target	Annual Price	Scope
Free	Individual patients & caregivers	€0	Personal checklist, alarms, education
Starter	Small programs (≤50 patients)	€2,999/year	Coordinator dashboard, templates, basic analytics

Tier	Target	Annual Price	Scope
Professional	Medium programs (≤200 patients)	€7,999/year	Care-team chat, FHIR integration, custom workflows
Enterprise	Large health systems / IDNs	€20K–€50K/year	Unlimited, SSO, SLA, dedicated CSM, custom integrations

Pricing rationale: - All competitors use opaque enterprise sales — transparent pricing is a trust differentiator - Starter tier is low enough to bypass committee approval in some centers - Enterprise priced 20–40% below equivalent generic platforms to win displacement

Freemium flywheel: Free patient app → viral coordinator invitations → trial → paid conversion

Slide 7: Traction & Pilot Readiness

Current state: - Existing Next.js codebase with patient/case/task workflows - Document upload & review pipeline - Dashboard & analytics views - Bootstrap 5.3 PWA-ready UI

Security & compliance gaps being closed: - Middleware JWT bypass (P0) — fix in progress - Hardcoded fallback secret (P0) — fix in progress - Rate limiting (P0) — implementation scheduled - SOC 2 Type II audit engagement: Month 3 target

Pilot strategy: - Target 1 academic + 1 community program for diversity - 90-day pilot with defined success metrics (checklist completion rate, coordinator time saved) - Manual contracts; no self-serve billing required for first revenue

Slide 8: Competitive Advantage — Why We Win

1. **Domain depth:** Only platform built for the 5-stage pre-transplant journey (consideration → center selection → evaluation/listing → waiting → offer/surgery)
2. **First-mover in the gap:** No competitor combines transplant-specific checklists + real-time patient-coordinator chat + document workflows
3. **Compliance-first architecture:** HIPAA-ready on day one; GDPR localization path; granular consent management (none of the generic competitors expose this)
4. **Integration, not replacement:** SMART on FHIR launch from Epic/Cerner reduces procurement friction vs. rip-and-replace
5. **Coordinator-first design:** Built for the daily pain point, not the CIO's checklist

Slide 9: Financial Projections — Three Scenarios

24-Month Revenue (Zero-Bootstrap Model):

Scenario	Y1 Revenue	Y2 Revenue	24-Mo Total
BEAR (30%)	€2,750	~€0	€2,750
BASE (50%)	€14,835	€98,838	€113,673
BULL (20%)	€83,688	€403,026	€486,714
Risk-adjusted expected			€155,005

Key assumptions: - Founder-only, zero paid marketing (CAC ≈ €0) - Infrastructure cost: €70–€120/month - Monthly churn: 3–8% (Starter/Pro); annual churn: 15–50% (Enterprise) - Time to first paying customer: 3–9 months

Unit economics (BASE): - Starter LTV: €5,000 | Professional LTV: €13,333 | Enterprise LTV: €144,000 - Cash CAC: €0 | LTV:CAC = infinite on cash basis

Slide 10: Regulatory Pathway — Compliance as Moat

Jurisdiction	Framework	Status
United States	HIPAA + HITECH	Architecture designed; BAA negotiations pending
European Union	GDPR (Article 9 special-category data)	DPIA scheduled Month 6; EU hosting architecture Month 9
Germany	BfArM DiGA fast-track	Feasibility assessment Month 12–18
United Kingdom	NHS Data Security Standards	Phase-3 expansion via NHSBT partnership

Compliance milestones: - Month 1: HIPAA Security Rule gap analysis - Month 2: Signed BAAs with cloud provider + sub-processors - Month 3: SOC 2 Type II auditor engagement - Month 6: GDPR DPIA complete - Month 12: SOC 2 Type II report issued

Why this matters: In healthcare SaaS, non-compliance is a terminal risk. A competitor with more features but no SOC 2 loses to a compliant vendor every time.

Slide 11: Team & Vision

Founder-led, domain-informed, compliance-first.

The team combines healthcare SaaS product experience with deep understanding of the transplant coordinator workflow. The technical stack (Next.js, PostgreSQL, Prisma, Bootstrap) is production-tested and evolvable.

Vision:

Every transplant patient, anywhere in the world, should have a clear, trackable path from referral to transplant — and every coordinator should know exactly where every patient stands, without opening a spreadsheet.

Near-term milestones: - Month 3: First pilot clinic live - Month 6: First paying customer - Month 12: SOC 2 Type II + 10 paying customers - Month 18: First Enterprise contract + FHIR read integration - Year 2: Multi-organ expansion + EU market entry

Slide 12: The Ask — Use of Funds

Seeking: Angel / Pre-Seed round to accelerate from zero-bootstrap to first 10 paying customers

Use of Funds	Allocation	Purpose
Security & compliance	30%	SOC 2 Type II audit, HIPAA gap remediation, penetration testing
Engineering	35%	FHIR integration, multi-tenant hardening, living-donor module
Sales & pilot support	20%	First CSM hire, pilot success metrics tracking, case-study production
Legal & operations	15%	BAAs, GDPR DPIA, incorporation, cyber insurance

Success metrics for this round: - 10+ paying customers across 3+ transplant programs - €5K+ MRR - Net revenue retention >100% - SOC 2 Type II clean report - 1 published pilot case study with time-to-listing improvement data

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All market data sourced from SRTR/OPTN, WHO, Eurotransplant, NHSBT, and live competitor website inspection. Financial projections are scenario-planning tools, not forecasts. Assumptions are explicitly stated in the Zero-Budget Financial Model (t_59df76ad).